

A FAITHFUL PATH COMMUNITY GUIDE

The Christian Spiritual Reset

*A 3-Day Guided Retreat to Overcome Burnout,
Hear God Clearly, and Renew Your Faith*

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FAITHFUL PATH COMMUNITY

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CHAPTER 16

When You Need a Pastor, Counsellor, or Mental-Health Professional

For any week, whenever it applies

Chapter 5 said, before the retreat, that a retreat cannot replace medical or psychological care. This chapter says it again, after the retreat, for a different reason. Before you went, the question was whether you were safe to go. Now the question is what the retreat showed you. Three days of quiet often make visible what months of noise had covered, and some readers will have come home knowing, perhaps for the first time, that what they are carrying needs more than rest and a rule of life.

If that is you, this chapter is not a verdict. It is a map.

Signs that the retreat surfaced something more

Chapter 4 described where pastoral description stops being adequate. Read it again now, in the light of what happened, and add these post-retreat signs.

The dip has not lifted. Session Ten said to expect a flat week. If the flatness has lasted most days for two weeks or more, or if it is deepening rather than easing, it may be more than readjustment. You do not need to wait two weeks if the symptoms are severe,

are rapidly worsening, are preventing ordinary functioning, or include thoughts of suicide or self-harm. Seek help now.

The grief remains overwhelming. Session Four may have brought grief or anger closer to the surface, and grief does not close neatly; ongoing sadness does not mean the session failed. But if it continues to overwhelm you, significantly disrupts ordinary life, sleep, or eating, or leaves you feeling unable to cope, additional support may help. A counsellor can be useful well before grief reaches any clinical threshold; you do not need to prove it is serious enough.

Something surfaced that frightens you. A memory, an experience from the past, a pattern of thought, a fear about yourself. If it affects your safety or daily life, or continues to intrude, consider taking it to an appropriately qualified person. You do not have to disclose everything immediately; you may begin by saying that something difficult surfaced and asking how confidentiality works.

You recognised yourself in Chapter 5. The sections on suicidal thoughts, abuse, substance dependence, behavioural addiction, or trauma. If you read them before the retreat and thought "not me," and after the retreat think "perhaps me," that shift is the point of this chapter.

The rule of life is not holding. Not because you are undisciplined, but because something underneath keeps knocking it over: anxiety that will not settle, a low mood that swallows the mornings, compulsions, panic, an inability to concentrate.

Or simply this: you are not getting better, and you have been faithful. Chapter 5 said that a lack of immediate relief does not mean the retreat failed. It is also true that faithfulness without

improvement, over weeks, is a reason to seek help, not a reason to try harder.

Who does what

Christians often go to the wrong person first, usually a pastor, because it is the person they know. Pastors are essential and this chapter will say so, but they are not the right first call for everything. Here is a rough guide. It is general; titles, qualifications, and how care is accessed vary by country, and the resources page at the back will help you find what exists where you live.

Need	Appropriate help	What they do
Spiritual dryness, questions of faith, guidance about a decision, church-related hurt, ongoing pastoral support	A pastor or spiritual director	Prayer, Scripture, spiritual counsel, discernment, community. Not diagnosis or treatment.
Grief, relationship difficulty, life transitions, moderate anxiety or low mood, patterns you want to understand	A qualified counsellor or therapist	Talking therapy. Many are Christian or respectful of faith; ask.
Persistent low mood, loss of interest, hopelessness, sleep and appetite changes, panic, intrusive thoughts, trauma symptoms, anything affecting daily functioning	A doctor or qualified mental-health professional, according to how care is accessed in your country. A doctor is especially important where physical illness, medication, substance withdrawal, or major sleep changes may be involved	Assessment, diagnosis where appropriate, and treatment, which may include therapy, medication, or both.

WHEN YOU NEED A PASTOR, COUNSELLOR, OR MENTAL-HEALTH PROFESSIONAL

Need	Appropriate help	What they do
Thoughts of death, suicide, or not wanting to be alive	A crisis line or urgent mental-health service now; emergency services or the nearest emergency department if you may act, have a plan, have access to the means, or cannot stay safe	Assess immediate risk, help you remain safe, and connect you with continuing care.
Abuse or coercive control, past or present	A specialist domestic-abuse or sexual-violence service, contacted from a safe device; emergency services if there is immediate danger and it is safe to call	Help you assess danger, make a safety plan, and consider housing, legal, medical, or police options without pressuring you. Joint counselling or a couples' retreat may be unsafe where abuse exists.
Substance dependence	A doctor and an addiction service; a recovery community	Safe withdrawal, treatment, ongoing recovery support. If you may be physically dependent on alcohol, sedatives, or another substance, do not stop abruptly on your own; seek medical advice about safe withdrawal.
Behavioural addiction	A counsellor with relevant experience; an accountable community	Therapy and accountability.

Need	Appropriate help	What they do
Scrupulosity, compulsive confession or checking, religious anxiety	A clinician experienced in OCD and scrupulosity, particularly cognitive behavioural therapy including exposure and response prevention where appropriate	Evidence-based treatment. Repeated reassurance, confession, and requests for spiritual certainty can become part of the compulsion; a pastor collaborating with the clinician should avoid reinforcing that cycle.
Serious misconduct, safeguarding concerns, legal exposure	The appropriate safeguarding, legal, or professional authority	Accountability, reporting, protection of others.

Two things the table implies. First, most serious difficulties need more than one kind of help, and a pastor is often one part of it rather than the whole. Second, the pastor's role is real. Someone who is receiving treatment for depression still needs prayer, Scripture, and a community that knows them. The mistake is not going to a pastor. It is going only to a pastor for something a pastor is not trained to treat.

Christian objections, answered briefly

Chapter 5 dealt with these, and they tend to return under pressure, so here they are again, shorter.

Seeking therapy means my faith is not enough. Faith does not require you to reject appropriate care. Luke, who wrote a Gospel, was a physician; Paul told Timothy to take a remedy for his stomach. Prayer, pastoral support, medicine, and therapy can work together.

Medication would be a spiritual defeat. Medication is a medical treatment, not a verdict on your faith. It may help some people and may not be appropriate for everyone. Discuss likely benefits, risks, alternatives, side effects, and review arrangements with a qualified prescriber, and do not start, stop, or change prescribed medication solely because of something you experienced on retreat.

A Christian should be able to pray this away. Continue to pray, and allow prayer to lead you toward truthful help rather than away from it. Prayer and professional treatment are not competitors. Receiving appropriate help may be one of the ways God provides care, and asking questions about that help is responsible rather than faithless. **Proverbs 15:22** says, "Without counsel plans fail, but with many advisers they succeed."

My pastor should be enough. A pastor can provide spiritual care, but responsible pastors recognise when medical, psychological, legal, addiction, or safeguarding expertise is needed. If yours does not, that is a reason to find additional help, not a reason to do without it.

People will find out. Professional care is generally confidential within the laws and ethical rules of your country. Ask the professional to explain confidentiality and its limits before you begin; you may start by asking questions without telling your whole story. In most circumstances, you decide what to tell your church, though safeguarding, legal, employment, or immediate-safety obligations may create exceptions.

How to choose a counsellor or therapist

Check legal registration or recognised professional credentials; in most countries there is a register you can consult. Ask about training relevant to what you are bringing, and about their

treatment approach and why they recommend it. Ask about experience with grief, trauma, burnout, anxiety, OCD, abuse, addiction, or clergy care, as applicable. Ask how they relate to faith; you do not need a Christian counsellor for good therapy, but you do need one who respects that your faith is central to you and will not treat it as the problem. Ask about confidentiality and its limits, costs and cancellation policies, expected review points, and how concerns or complaints are handled.

After two or three sessions, ask yourself: do I feel respected, listened to, and able to raise concerns? Therapy may sometimes feel emotionally difficult without being unsafe. If you do not feel respected or heard, it is acceptable to change. Both the relationship and an appropriate, evidence-based approach matter.

If cost is a barrier, ask about sliding scales, charitable counselling services, church-funded schemes, employee assistance programmes, university training clinics, and public health provision. Chapter 14 said not to let an unavailable appointment become no action; the same applies to an expensive one.

For pastors and leaders

Program 7 said it: find help outside your own church. A minister in counselling with a member of their own congregation, or receiving pastoral care from someone they lead, is rarely free to be honest, and the dual relationship is unfair to both people. Use your denomination's provision if it has one, a professional supervisor, a counsellor with experience of clergy, or a pastor in another town.

Consider who, if anyone, in your leadership needs to know that you are receiving support. Follow safeguarding, professional, denominational, and employment requirements, but keep clinical

details private. If disclosure could place you at risk of unfair treatment, seek confidential professional or denominational advice first. If your tradition has a culture in which ministers are not allowed to be unwell, you may be the person who begins to change it, but you do not have to do that by exposing yourself to harm.

For those in a crisis of faith

Some readers will have come home from the retreat with something they did not expect: the discovery that they are not sure what they believe, or whether they believe. This is not a mental-health condition, and it is not necessarily a sign that anything is wrong with you. It is often what happens when a faith that was carried by activity finally stops moving and has to stand on its own.

Do not carry it alone, and do not carry it to someone who will panic. A pastor outside your own church, a spiritual director, a mature believer who has walked through doubt themselves; these are the people to talk to. Keep a gentle connection with God if you can, but do not turn prayer or Scripture into a test of whether you still believe. If these practices trigger compulsions or trauma responses, seek guidance about adapting them safely. And be patient. Faith that is allowed to ask honest questions can become more mature and less dependent on performance, though the process may be slow and its outcome should not be forced.

If you are in danger

Everything in this chapter assumes you have time to choose. If you do not, the map collapses to one instruction: get help immediately.

If you may act on suicidal thoughts, have a plan or access to the means, cannot keep yourself safe, or are in immediate danger from another person, contact emergency services or go to the nearest emergency department now. Use a crisis or abuse service from a safe device. Move to a safer or public place if possible, tell a trusted person, and do not remain alone. If it is safe, create distance between yourself and anything you could use to harm yourself.

If you do not know the crisis number in your country, search for your national emergency service or crisis line, or ask a hospital, doctor, or trusted person to help you make contact. The resources page at the back explains how to find these services, but do not wait to turn to it if you need help now. The rest of this book can wait.

Before Chapter 17

1. Did any of the post-retreat signs describe you? Which?
2. Using the table, who is the right first call for what you are carrying? Write the kind of help, not yet the name.
3. Which of the objections has been holding you back, if any? Write the answer to it in your own words.
4. What is the one step this week: a call to your doctor, an enquiry to a counsellor, a conversation with a pastor outside your church, a crisis call today? Write it, and do it.